

How Are We Getting Paid?

A Plain-Language Look at Our Billing & Collections

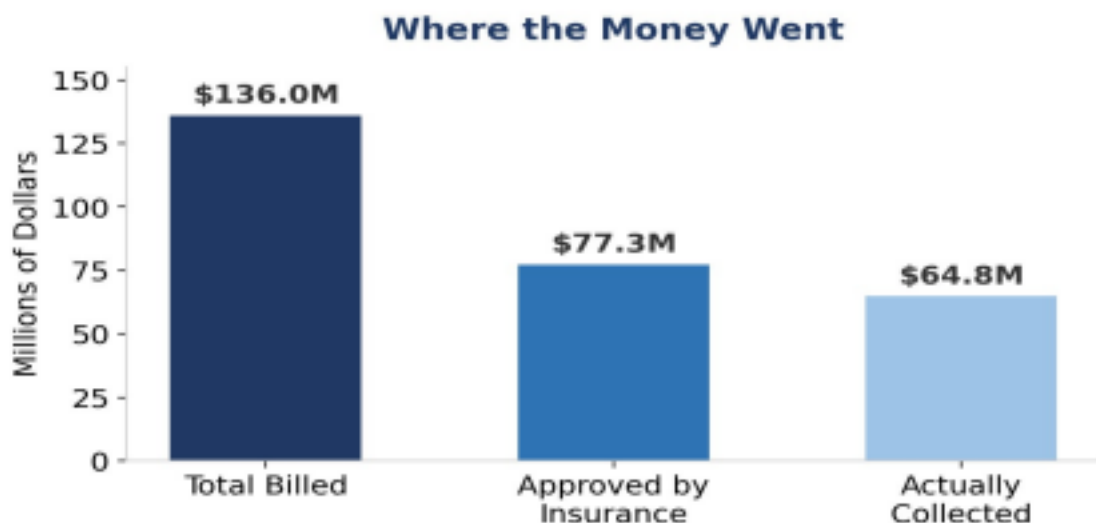
Prepared by: Abdul Oladosu

Covers: 20,000 patient visits, January 2024 – December 2025

Date: July 2026

The Big Picture

For every \$100 the hospital billed over the last two years, insurance companies agreed to pay about \$57 — and we actually collected about \$48 of that \$100. That gap between what's billed, what's approved, and what's actually collected is where this report focuses.

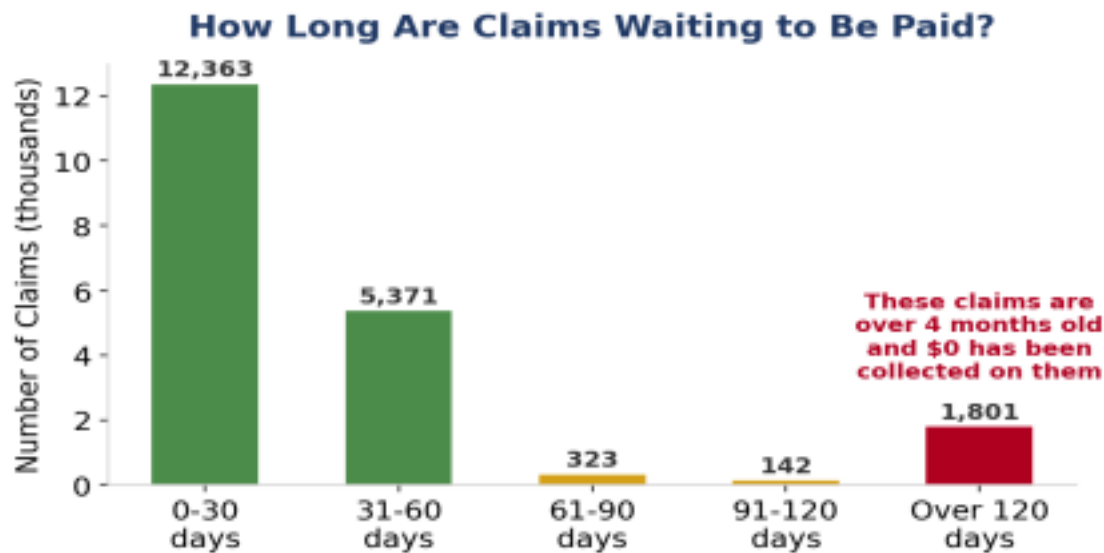


The good news: money is coming in, and claims are getting processed faster than they used to. The concerning news: there are two specific leaks costing the hospital real money, and a small number of avoidable mistakes are causing most of our claim denials.

In plain numbers: about 11 out of every 100 claims get denied by insurance, and about 35 out of every 100 paid claims are paid less than insurance actually approved. On top of that, \$12.3 million in old claims have been waiting more than 4 months with nothing collected.

Finding 1: Old Claims Are Turning Into Losses

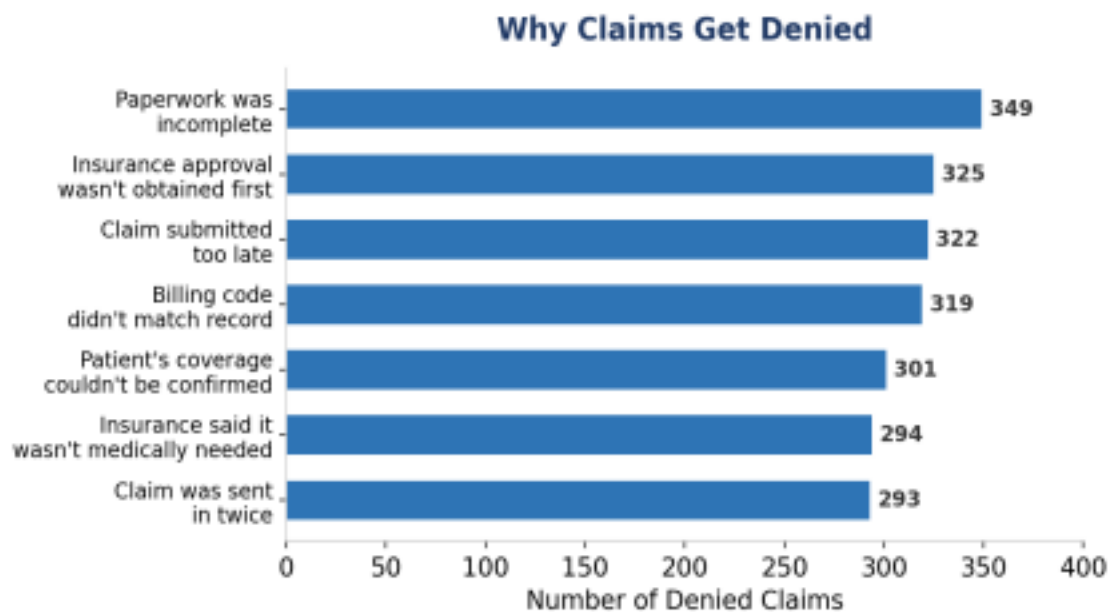
When a claim isn't paid within 120 days (about 4 months), it's very unlikely to ever get paid. Right now we have 1,801 claims — worth \$12.3 million in charges — sitting in that oldest, most at-risk category. Almost all of them (1,797 out of 1,801) have already been denied by insurance.



Think of it like a store return window: the longer a claim sits unresolved, the less likely we are to ever collect on it. Claims in the first month (0-30 days) are healthy — most get paid. But once a claim crosses the 120-day mark, it's essentially frozen — those are dollars we should assume are gone unless someone actively fights for them.

Finding 2: A Few Simple Mistakes Cause Most Denials

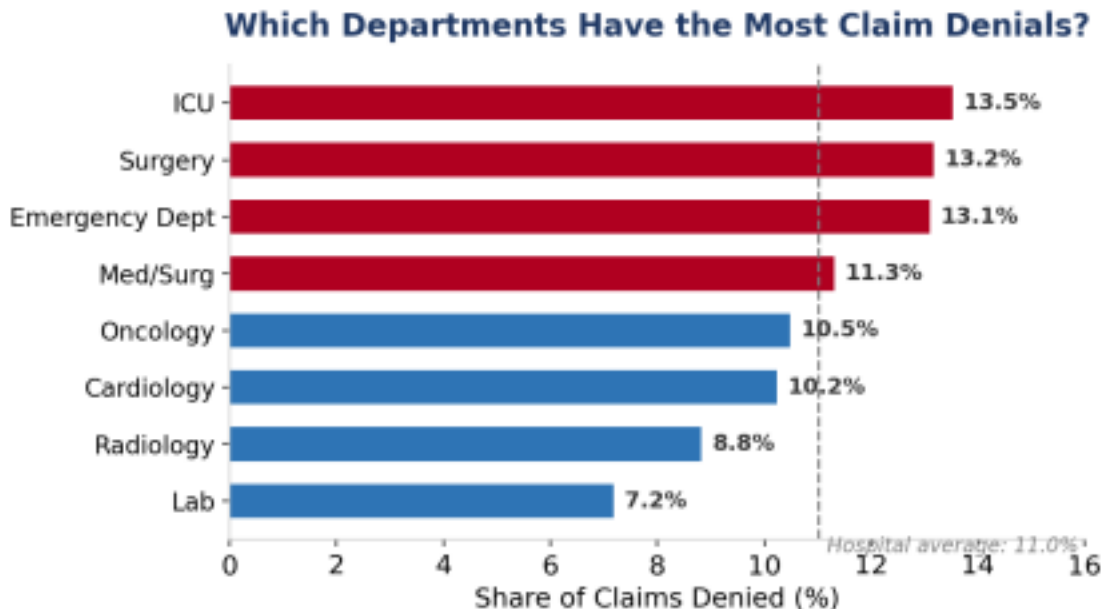
When insurance denies a claim, it's almost always for one of seven reasons — and they're all roughly equally common, which means there isn't one big villain to fix. It's several small, fixable problems adding up.



The top three reasons — incomplete paperwork, missing insurance approval, and claims sent in too late — all happen before the claim ever reaches the insurance company. In other words, most denials are preventable with better checks on our end, before we even submit the bill.

Some Departments Struggle More Than Others

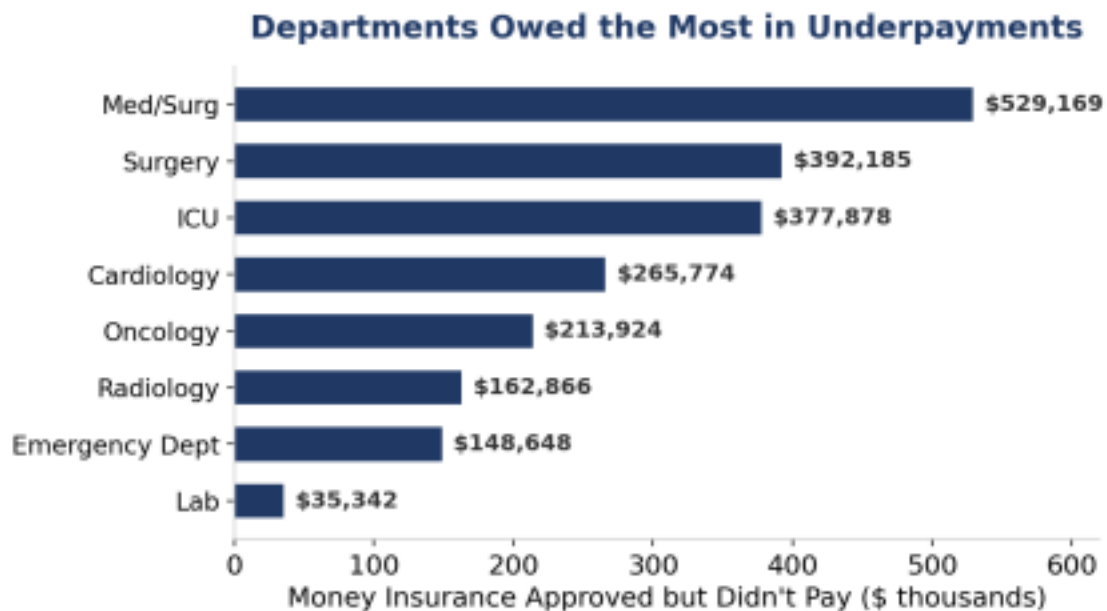
The ICU, Surgery, and Emergency Department have the highest denial rates — all above the hospital's overall average of 11%. Lab and Radiology, by comparison, do noticeably better.



This pattern makes sense: ICU, Surgery, and ED handle the most complex, fast-moving cases, where paperwork and pre-approvals are easiest to miss. These are the departments where extra front-end checks would have the biggest payoff.

Finding 3: Insurance Is Paying Us Less Than It Approved

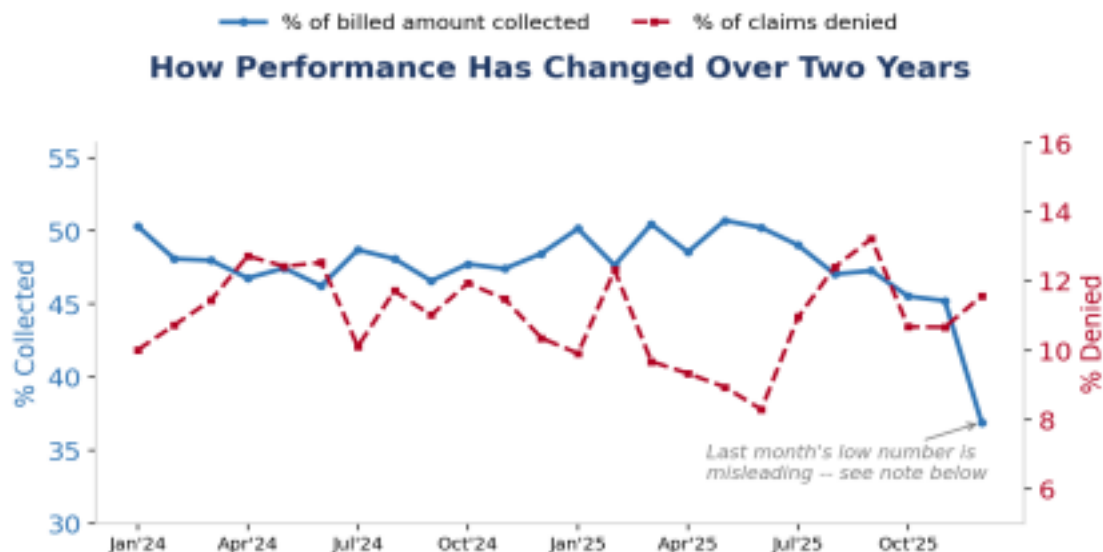
This is a separate problem from denials, and it's easy to miss because it doesn't show up as a "denied" claim at all. In these cases, insurance approves an amount — but then pays us less than that approved amount. Over the two years studied, this happened on 7,087 claims, adding up to \$2.1 million that was approved but never collected.



Med/Surg, Surgery, and the ICU account for about 6 out of every 10 dollars in this gap. Because insurance already agreed to pay this money, recovering it is usually just a matter of following up with the payer — it doesn't require appealing a denial.

Finding 4: We're Getting Faster, With One Caveat

Over the two years, the time it takes to resolve a claim has steadily improved — from about 60 days on average down to under 30 days by late 2025. That's a real, meaningful improvement worth protecting.



About the dip in December 2025: the collection number for the most recent month looks unusually low (37% instead of the usual 47-50%). This is most likely because many December claims simply haven't had enough time to be processed yet — not because something suddenly went wrong. We recommend re-checking this specific number in a couple of months once those claims have had time to resolve, rather than reacting to it now.

What We Recommend Doing About It

1. Deal With the Old, Stuck Claims First

- Sort the 1,801 old claims into two piles: the 4 still awaiting a decision (follow up immediately) and the 1,797 already denied (decide quickly whether to appeal or write off).
- Set a rule: any claim approaching 90 days without payment gets a decision — appeal it or close it — within 10 business days, so claims stop drifting into the unrecoverable pile.

2. Catch Mistakes Before the Claim Goes Out the Door

- Add a simple pre-submission checklist in ICU, Surgery, and the ED — the three departments with the most denials — to catch missing paperwork and coding mismatches before claims are sent.
- Double-check insurance approval and eligibility at check-in, especially for self-pay and Medicaid patients, where this kind of denial is most common.

3. Go Collect the Money Insurance Already Approved

- Start an underpayment follow-up effort focused on Med/Surg, Surgery, and ICU claims first — that's where 61% of the \$2.1 million gap is sitting.
- Start tracking this underpayment gap as its own number every month, separate from denials, so it doesn't stay invisible the way it has been.

4. Don't Overreact to Last Month's Numbers

- Wait roughly 60-90 days before treating the December 2025 dip as a real problem, and re-check the number once those claims have finished processing.
- Going forward, track what share of each month's claims are still "in progress" alongside the headline numbers, so a partly-finished month doesn't look like a bad month.

Bottom Line

None of these problems require a big new system or a large investment — they're process fixes. Tightening paperwork checks in three departments, actively working the oldest claims, and following up on approved but-underpaid claims could recover several million dollars a year without changing how care is delivered.